

Policy Template Library

18 Essential Policy Templates for ASC Compliance

Product: Ambulatory Surgery Center Compliance Manual — Tier 2

Version: 1.0

Applicable Standards: CMS Conditions for Coverage (42 CFR Part 416), AAAHC Standards, OSHA, HIPAA, State Requirements

How to Use This Library

Each policy template below includes:

- **Policy statement** and purpose
- **Scope** of who it applies to
- **Key procedure steps** that meet regulatory requirements
- **Documentation requirements**
- **Review schedule**

Customization Instructions

1. Replace all **[FACILITY NAME]** placeholders with your ASC name
2. Replace **[DATE]** placeholders with the effective date
3. Review each policy against your state-specific requirements
4. Have your Governing Body approve each policy before implementation
5. Train all applicable staff on each policy
6. File the original signed policy and distribute copies as needed

Critical: These templates provide a compliant framework. You **MUST** customize them to reflect your facility's actual practices, equipment, staffing, and state regulations. A policy that doesn't match what you actually do is worse than no policy — surveyors will cite you for failing to follow your own policies.

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Policy 1: Infection Control Program

[FACILITY NAME] — Policy & Procedure

Field	Details
Policy Title	Infection Control Program
Policy Number	IC-001
Department	Facility-Wide
CMS Reference	42 CFR §416.51
AAAHHC Reference	Chapter 9 — Infection Prevention and Control and Safety
Effective Date	[DATE]
Last Reviewed	[DATE]
Next Review Due	[DATE + 12 months]
Approved By	Governing Body — Date: ____

Purpose

To establish a comprehensive infection prevention and control program that minimizes the risk of healthcare-associated infections (HAIs) to patients, staff, and visitors of **[FACILITY NAME]**.

Scope

This policy applies to all staff, credentialed providers, vendors, and visitors within **[FACILITY NAME]**.

Definitions

- **HAI:** Healthcare-Associated Infection — an infection acquired during the course of receiving healthcare treatment
- **IC Officer:** The designated individual responsible for the facility's infection control program
- **Standard Precautions:** Infection prevention practices applied to all patient care regardless of suspected or confirmed infection status
- **SSI:** Surgical Site Infection

Policy Statement

[FACILITY NAME] maintains an active infection prevention and control program that includes surveillance, prevention, and control of infections. The program is overseen by a qualified Infection Control Officer and is reviewed by the Governing Body at least annually.

Procedure

1. Infection Control Officer

- The IC Officer is appointed by the Governing Body
- Qualifications: Licensed healthcare professional with training/certification in infection control (or actively working toward certification)
- Responsibilities include:
 - Developing and implementing IC policies
 - Conducting infection surveillance
 - Reporting infections per state and federal requirements
 - Providing IC education to staff
 - Conducting annual IC risk assessment
 - Presenting IC data to QAPI committee and Governing Body

2. Surveillance Program

- Active surveillance for surgical site infections (SSIs) is conducted for all procedures
- Post-discharge SSI surveillance is conducted for a minimum of 30 days post-procedure (or per CDC/NHSN definitions)
- Surveillance data is collected monthly, analyzed quarterly, and reported to QAPI
- Infection rates are benchmarked against national data (NHSN) when available

3. Prevention Measures

- Standard Precautions are followed for all patient encounters
- Transmission-based precautions are implemented when indicated
- Surgical site infection prevention bundles include:
 - Appropriate preoperative skin antisepsis
 - Appropriate antibiotic prophylaxis (per SCIP/current guidelines)
 - Normothermia maintenance
 - Glucose control (diabetic patients)
 - Appropriate hair removal (no razors)
- Environmental cleaning per established schedules and using EPA-registered disinfectants
- Sterile processing per AAMI standards and manufacturer IFU

4. Reporting

- Reportable infections are reported to the state health department per state law
- SSI data is reported to NHSN (if participating)

- All infection data is reported to the QAPI committee quarterly
- IC annual report is presented to the Governing Body

5. Annual Risk Assessment

- The IC Officer conducts an annual infection control risk assessment
- The risk assessment evaluates: patient population, procedures performed, geographic risks, prior infection data, emerging pathogens, construction/renovation activities
- Priorities are established based on risk assessment findings
- The risk assessment is approved by the Governing Body

Documentation

- Infection surveillance logs (maintained monthly)
- IC committee meeting minutes (quarterly minimum)
- Annual IC risk assessment
- IC Officer appointment letter and qualifications
- Staff IC education records
- Reportable infection submissions

Review

This policy is reviewed annually by the IC Officer and approved by the Governing Body.

Policy 2: Hand Hygiene

[FACILITY NAME] — Policy & Procedure

Field	Details
Policy Title	Hand Hygiene
Policy Number	IC-002
Department	Facility-Wide
CMS Reference	42 CFR §416.51
Effective Date	[DATE]
Last Reviewed	[DATE]
Next Review Due	[DATE + 12 months]
Approved By	Governing Body — Date: ____

Purpose

To reduce the transmission of healthcare-associated pathogens through consistent and proper hand hygiene practices per CDC and WHO guidelines.

Scope

All staff, credentialed providers, vendors, and visitors of [FACILITY NAME].

Policy Statement

Hand hygiene is the single most effective measure to prevent healthcare-associated infections. All personnel are required to perform hand hygiene per the CDC/WHO 5 Moments of Hand Hygiene.

Procedure

1. Five Moments of Hand Hygiene

1. Before patient contact
2. Before aseptic task
3. After body fluid exposure risk
4. After patient contact
5. After contact with patient surroundings

2. Acceptable Methods

- **Alcohol-based hand rub (ABHR):** Preferred method when hands are not visibly soiled. Apply product, rub all surfaces of hands until dry (minimum 20 seconds)
- **Soap and water:** Required when hands are visibly soiled, after caring for patients with C. difficile or norovirus, and after using the restroom. Wet hands, apply soap, rub all surfaces for minimum 20 seconds, rinse, dry with disposable towel
- **Surgical hand scrub:** Required before all surgical procedures per manufacturer IFU for the surgical scrub product in use

3. Hand Hygiene Supplies

- ABHR dispensers are available at all points of care, room entrances/exits, and common areas
- Soap dispensers and paper towels are available at all sinks
- Supplies are checked and restocked daily

4. Artificial Nails & Jewelry

- Artificial nails, nail extenders, and nail wraps are prohibited for all staff with direct patient contact
- Natural nails are kept short (< ¼ inch beyond fingertip)
- Ring policy: [FACILITY: define your policy — e.g., plain wedding band only, or no rings in clinical areas]

5. Monitoring & Compliance

- Hand hygiene compliance is monitored through direct observation a minimum of quarterly
- Compliance rates are reported to QAPI and IC committee
- Target compliance rate: ≥ 90%
- Non-compliance is addressed through individual counseling and re-education

6. Education

- Hand hygiene training is provided at orientation and annually
- Competency includes return demonstration of proper technique

Documentation

- Hand hygiene observation audit forms
 - Quarterly compliance rates
 - Staff training records
-

Policy 3: Sterilization & High-Level Disinfection

[FACILITY NAME] — Policy & Procedure

Field	Details
Policy Title	Sterilization & High-Level Disinfection
Policy Number	IC-003
Department	Sterile Processing / OR
CMS Reference	42 CFR §416.51
Effective Date	[DATE]
Last Reviewed	[DATE]
Next Review Due	[DATE + 12 months]
Approved By	Governing Body — Date: ____

Purpose

To ensure all reusable surgical instruments and devices are properly decontaminated, sterilized, or high-level disinfected per manufacturer IFU, AAMI standards, and CDC guidelines.

Policy Statement

All reusable critical devices (items that enter sterile tissue) are sterilized before use. All reusable semi-critical devices (items that contact mucous membranes) undergo high-level disinfection (HLD) or sterilization. Processing follows manufacturer IFU, AAMI ST79 (steam sterilization), and AAMI ST91 (flexible endoscope reprocessing) as applicable.

Procedure

1. Decontamination

- Used instruments are transported to decontamination in closed, labeled containers
- Manual cleaning per manufacturer IFU using appropriate enzymatic detergent
- Automated cleaning (ultrasonic and/or washer-disinfector) per manufacturer IFU
- Visual inspection of each instrument for cleanliness, function, and damage after cleaning

2. Packaging & Loading

- Instruments are packaged in FDA-cleared sterilization packaging
- Internal chemical indicator placed in each package
- External chemical indicator visible on outside of each package
- Sterilizer loaded per manufacturer specifications (not overloaded)

3. Steam Sterilization

- Sterilization parameters follow manufacturer IFU for the sterilizer and the instruments being processed
- Minimum: 270°F (132°C) for 4 minutes prevacuum, or per specific instrument IFU
- Biological indicator (BI) testing: performed at least weekly and with every implant load
- Chemical indicators (CI): internal and external CI with every load

- Bowie-Dick test (prevacuum sterilizers): daily, before first processed load
- All sterilizer printouts are reviewed, initialed, and filed

4. Biological Indicator Failure Protocol

- If a BI test is positive:
 - a. Immediately quarantine all items processed since the last negative BI
 - b. Notify IC Officer and administration
 - c. Remove sterilizer from service
 - d. Investigate root cause (operator error, mechanical failure, BI placement)
 - e. Run three consecutive negative BIs before returning sterilizer to service
 - f. Recall any affected items that were used on patients — notify patients per policy
 - g. Document all actions taken

5. Immediate-Use Steam Sterilization (IUSS / Flash)

- IUSS is used only when there is an urgent patient need and no other sterile instrument is available
- IUSS is NOT used for implants
- IUSS is NOT used for convenience, routine use, or to compensate for insufficient instrument inventory
- Each IUSS cycle is documented with patient name, reason for IUSS, and cycle parameters
- IUSS usage is tracked and reported to QAPI quarterly

6. High-Level Disinfection (HLD)

- Semi-critical devices undergo HLD per manufacturer IFU
- HLD solution concentration is tested before each use (minimum effective concentration — MEC)
- Contact time follows manufacturer IFU (minimum per EPA label)
- Rinsing per IFU with appropriate water quality
- Drying and storage per IFU and AAMI ST91

7. Single-Use Devices

- Single-use devices are NOT reprocessed at **[FACILITY NAME]** unless using an FDA-cleared third-party reprocessor
- Single-use devices are discarded after one use

8. Storage

- Sterile items are stored in a clean, dry, controlled environment
- Items are stored off the floor, away from ceiling, and in covered/enclosed cabinets
- Event-related sterility: items are considered sterile until the package is compromised
- Staff visually inspect package integrity before use

Documentation

- Sterilizer load records (each cycle)
 - BI test results (weekly minimum + all implant loads)
 - Bowie-Dick test results (daily for prevacuum)
 - HLD records (each cycle)
 - IUSS usage log
 - Equipment maintenance records
 - Staff competency records for sterile processing
-

Policy 4: Medication Management

[FACILITY NAME] — Policy & Procedure

Field	Details
Policy Title	Medication Management
Policy Number	PHARM-001
Department	Facility-Wide
CMS Reference	42 CFR §416.48
Effective Date	[DATE]
Last Reviewed	[DATE]
Next Review Due	[DATE + 12 months]
Approved By	Governing Body — Date: ____

Purpose

To ensure the safe ordering, storage, administration, and monitoring of medications within [FACILITY NAME].

Policy Statement

Medications are managed through a safe, organized system that minimizes the risk of medication errors. All medications are administered by qualified, licensed personnel per physician orders.

Procedure

1. Ordering

- Medications are ordered by authorized prescribers only (licensed and credentialed at the facility)
- Verbal/telephone orders are accepted only by qualified licensed personnel, read back for verification, and authenticated by the prescriber within 48 hours
- Pre-printed order sets are reviewed and approved by the Medical Director annually

2. Storage

- Medications are stored per manufacturer specifications
- Refrigerated medications: 36–46°F (2–8°C); temperature logged daily (or continuously monitored with alarm)
- Medications are secured; medication storage areas are locked when unattended
- Look-alike/sound-alike medications are physically separated or flagged
- No medications are stored in unlocked areas accessible to unauthorized personnel

3. Administration

- Five Rights verified before every administration: Right Patient, Right Drug, Right Dose, Right Route, Right Time
- Patient identity confirmed with two identifiers before administration
- Allergies verified before administration
- All administered medications are documented immediately in the patient record

4. Multi-Dose Vials

- Multi-dose vials are labeled with the date opened
- Multi-dose vials are discarded 28 days after opening (or per manufacturer IFU if shorter)
- Multi-dose vials are dedicated to a single patient whenever possible
- Multi-dose vials are stored per manufacturer IFU (refrigeration if required)

5. Single-Dose Vials

- Single-dose vials are used for one patient only and discarded immediately after use
- Single-dose vials are NEVER entered more than once, even for the same patient
- Remaining medication in a single-dose vial is discarded; it is NOT saved for later use

6. Expiration Monitoring

- Medication expiration dates are checked monthly (minimum)
- Expired medications are removed immediately and disposed of per policy
- Medications approaching expiration (within 30 days) are flagged

7. Adverse Drug Events

- All adverse drug events, medication errors, and near-misses are reported per the Incident Reporting policy
- Significant adverse events are reported to the physician, patient, and QAPI committee
- Root cause analysis is performed for serious medication events

Documentation

- Medication administration records (each patient)
 - Temperature logs (daily for all medication storage units)
 - Monthly expiration check logs
 - Controlled substance records (see Policy 5)
 - Medication error/adverse event reports
 - Pharmacy consultant reports (if applicable)
-

Policy 5: Controlled Substance Management

[FACILITY NAME] — Policy & Procedure

Field	Details
Policy Title	Controlled Substance Management
Policy Number	PHARM-002
Department	Facility-Wide
CMS Reference	42 CFR §416.48 / DEA 21 CFR 1301-1321
Effective Date	[DATE]
Last Reviewed	[DATE]
Next Review Due	[DATE + 12 months]
Approved By	Governing Body — Date: ____

Purpose

To ensure the secure ordering, receipt, storage, administration, documentation, waste, and disposal of controlled substances in compliance with DEA regulations and state pharmacy law.

Policy Statement

Controlled substances are managed under strict accountability. All controlled substances are double-locked, counted at each shift change, and discrepancies are investigated immediately.

Procedure

1. Registration & Authorization

- [FACILITY NAME] maintains a current DEA registration (posted in a visible location)
- Only authorized personnel with DEA registration or under the facility's DEA registration may order controlled substances
- State-specific controlled substance licenses are maintained as required

2. Ordering & Receipt

- Controlled substances are ordered using DEA Form 222 (Schedule II) or electronic ordering via CSOS
- All orders are received by two authorized personnel
- Received quantities are verified against the order and documented immediately
- Invoice copies are retained for minimum 2 years

3. Storage

- All controlled substances are stored in a double-locked system (lock within a lock, or two separate locking mechanisms)
- Keys or access codes are limited to authorized personnel
- Storage areas are not accessible to unauthorized individuals

4. Administration & Documentation

- Controlled substances are administered only per physician order

- Administration is documented on the controlled substance log AND the patient record
- Documentation includes: date, time, patient name, drug name, dose, route, administering person, and physician name
- Waste is witnessed by a second licensed person and documented with both signatures

5. Counting

- Controlled substance perpetual inventory count is performed at each shift change (minimum) and at the beginning and end of each operative day
- Counts are documented with signature of outgoing and incoming staff
- Any discrepancy triggers immediate investigation

6. Discrepancy Resolution

- If a count discrepancy is identified:
 - a. Immediate recount by both parties
 - b. Review all documentation for the period since last accurate count
 - c. Notify the Administrator/designee immediately
 - d. If unresolved within 24 hours, notify the DEA (if required) and follow state reporting requirements
 - e. Document all investigation steps and outcome
 - f. Report to QAPI committee

7. Waste & Disposal

- Controlled substance waste is witnessed and co-signed
- Disposal of outdated or unused controlled substances follows DEA regulations (reverse distributor or authorized destruction)
- Disposal is documented and records retained for minimum 2 years

Documentation

- Controlled substance perpetual inventory log
 - Shift count verification sheets
 - DEA Form 222 and invoices
 - Waste documentation (dual-signed)
 - Discrepancy investigation reports
 - DEA registration (current, posted)
-

Policy 6: Patient Rights & Responsibilities

[FACILITY NAME] — Policy & Procedure

Field	Details
Policy Title	Patient Rights & Responsibilities
Policy Number	PR-001
Department	Facility-Wide
CMS Reference	42 CFR §416.50
Effective Date	[DATE]
Last Reviewed	[DATE]
Next Review Due	[DATE + 12 months]
Approved By	Governing Body — Date: ____

Purpose

To ensure that all patients at **[FACILITY NAME]** are informed of their rights and responsibilities, and that these rights are respected and protected throughout their care.

Policy Statement

Every patient has the right to be informed of their rights prior to the date of their procedure. **[FACILITY NAME]** provides a Patient Rights notice to each patient (or patient representative) and posts this notice in a visible location within the facility.

Patient Rights

Patients of **[FACILITY NAME]** have the right to:

1. **Be informed of their rights** prior to the day of their procedure, in a language they understand
2. **Receive care in a safe setting** free from all forms of abuse, neglect, and harassment
3. **Be treated with respect and dignity** with recognition of their personal privacy
4. **Be free from discrimination** based on race, color, national origin, sex, age, disability, religion, sexual orientation, or gender identity
5. **Receive information** about the procedure, risks, benefits, alternatives, and the identity of their physician and other caregivers
6. **Give or refuse informed consent** before any procedure or treatment
7. **Formulate advance directives** and have them followed by the facility
8. **Be informed of any experimental treatments** and to refuse participation without jeopardizing care
9. **Voice complaints and grievances** without fear of discrimination or reprisal
10. **Receive a prompt response** to complaints and grievances
11. **Privacy and confidentiality** of their medical records per HIPAA
12. **Access their medical records** within a reasonable timeframe

- 13. **Be informed of their financial obligations** before treatment
- 14. **Receive discharge instructions** including follow-up care, medications, and warning signs

Patient Responsibilities

Patients of **[FACILITY NAME]** have the responsibility to:

- 1. Provide accurate and complete health information
- 2. Follow the treatment plan recommended by their physician
- 3. Inform staff of any changes in their condition
- 4. Arrange for a responsible adult for transportation and post-procedure care
- 5. Follow facility rules and regulations
- 6. Treat staff and other patients with respect
- 7. Meet financial obligations in a timely manner

Procedure

- Patient Rights notice is provided to each patient (or representative) prior to the procedure
- Patient Rights notice is posted in the reception/waiting area
- Patient acknowledges receipt of rights information (signature documented)
- Rights are provided in the patient’s preferred language when possible; interpreter services are available
- Staff are trained on patient rights annually

Documentation

- Signed patient acknowledgment of rights
- Posted notice in facility
- Staff training records

Policy 7: Informed Consent

[FACILITY NAME] — Policy & Procedure

Field	Details
Policy Title	Informed Consent
Policy Number	PR-002
Department	Clinical
CMS Reference	42 CFR §416.50
Effective Date	[DATE]
Last Reviewed	[DATE]
Next Review Due	[DATE + 12 months]
Approved By	Governing Body — Date: ____

Purpose

To ensure that patients give voluntary, informed consent before any procedure or anesthesia administration, based on a clear understanding of the proposed treatment.

Policy Statement

Informed consent is obtained by the physician performing the procedure and/or the anesthesia provider. The informed consent discussion and the signed consent form are documented in the patient's medical record before the procedure begins.

Required Elements of Informed Consent

The informed consent discussion and form must include:

1. **Name of the procedure** in language the patient understands
2. **Name of the physician** performing the procedure
3. **Description of the procedure** in understandable terms
4. **Risks and potential complications** including those specific to the procedure
5. **Benefits of the procedure**
6. **Alternatives** to the proposed procedure, including the option of no treatment
7. **Type of anesthesia** to be used and associated risks
8. **Patient's acknowledgment** that they understand and agree to proceed
9. **Patient's signature** (or legal representative) with date and time
10. **Physician's signature** with date and time
11. **Witness signature** with date and time

Procedure

- Informed consent is obtained by the physician in a private setting, allowing adequate time for questions
- Consent is obtained BEFORE the patient receives any sedation or pre-operative medication that could impair judgment
- If the patient cannot give consent (minor, incapacitated), consent is obtained from the legal representative
- If an interpreter is used, this is documented on the consent form
- The original signed consent is placed in the patient's medical record
- Consent for the procedure AND consent for anesthesia are obtained (may be combined or separate forms per facility preference)

Special Circumstances

- **Emergency:** If an emergency arises during the procedure requiring a different or additional procedure, the physician documents the medical necessity
 - **Refusal:** If a patient refuses consent, document the refusal, the information provided, and that the patient understands the consequences
 - **Withdrawal:** A patient may withdraw consent at any time before the procedure
-

Policy 8: Advance Directives

[FACILITY NAME] — Policy & Procedure

Field	Details
Policy Title	Advance Directives
Policy Number	PR-003
Department	Clinical
CMS Reference	42 CFR §416.50(a)(2)
Effective Date	[DATE]
Last Reviewed	[DATE]
Next Review Due	[DATE + 12 months]
Approved By	Governing Body — Date: ____

Purpose

To comply with the Patient Self-Determination Act by informing patients of their right to formulate advance directives and to establish a process for managing advance directives within the ASC setting.

Policy Statement

[FACILITY NAME] informs all patients of their right to formulate advance directives per state law. The facility documents whether the patient has an advance directive and, if so, obtains a copy for the medical record. In the ASC setting, the primary focus is safe completion of the planned procedure with transfer to an appropriate facility if the patient's condition deteriorates.

Procedure

1. During pre-operative assessment, staff asks each patient whether they have an advance directive
2. If yes: a copy is requested and placed in the medical record; the physician and anesthesia provider are informed
3. If no: the patient is provided with information about advance directives and resources to create one (the ASC does not create advance directives)
4. The physician discusses with the patient how the advance directive will be applied in the ASC setting — particularly that:
 - The primary goal is safe management during and after the procedure
 - Resuscitation and stabilization will be initiated in the event of an emergency during the procedure to safely complete the procedure
 - If a patient's condition deteriorates beyond the ASC's capability, transfer to a hospital will be initiated
5. The patient's wishes are documented and communicated to all members of the care team
6. If a patient wishes to suspend their advance directive during the procedure, this is documented

Policy 9: Grievance & Complaint Resolution

[FACILITY NAME] — Policy & Procedure

Field	Details
Policy Title	Grievance & Complaint Resolution
Policy Number	PR-004
Department	Facility-Wide
CMS Reference	42 CFR §416.50(a)(3)
Effective Date	[DATE]
Last Reviewed	[DATE]
Next Review Due	[DATE + 12 months]
Approved By	Governing Body — Date: ____

Purpose

To establish a process for receiving, investigating, and resolving patient complaints and grievances in a timely manner.

Definitions

- **Complaint:** A verbal or informal expression of dissatisfaction that can be resolved promptly at the point of service
- **Grievance:** A formal, written complaint or any complaint regarding abuse, neglect, patient harm, or Medicare billing that requires investigation

Procedure

1. Receiving Complaints/Grievances

- Any staff member may receive a complaint; all complaints are taken seriously
- Verbal complaints are documented and the staff member attempts immediate resolution
- If the complaint cannot be resolved immediately or the patient requests a formal process, it is escalated to a grievance
- Patients are informed of how to file a grievance (verbally and via posted notice)
- Grievance forms are available at the front desk and provided upon request

2. Investigation

- All grievances are logged in the Grievance Log with date received, patient name, nature of complaint, and assigned investigator
- Investigation is initiated within 3 business days
- The investigator interviews relevant staff, reviews medical records, and gathers facts
- Investigation is completed within 14 calendar days (or the patient is notified of the delay and expected resolution date)

3. Resolution & Response

- Written response to the patient includes: summary of investigation, actions taken, and who to

contact for further concerns

- If the grievance involves quality of care, it is referred to QAPI committee
- If the grievance involves a physician, it is referred to the Medical Director

4. External Reporting

- Patients are informed of their right to file a complaint with:
- The State Health Department: [STATE CONTACT INFO]
- The accrediting body (AAAHC / Joint Commission / CMS): [ACCREDITOR CONTACT]
- Medicare Beneficiary and Family Centered Care Quality Improvement Organization (BFCC-QIO): [QIO CONTACT]
- This information is posted in the facility and included in patient rights materials

5. Tracking & Trending

- All grievances are reviewed quarterly by the QAPI committee
- Trends are identified and systemic issues are addressed through performance improvement projects

Documentation

- Grievance log
- Investigation reports
- Written response letters
- QAPI committee minutes reflecting grievance review

Policy 10: Emergency Preparedness Plan

[FACILITY NAME] — Policy & Procedure

Field	Details
Policy Title	Emergency Preparedness Plan
Policy Number	EMRG-001
Department	Facility-Wide
CMS Reference	42 CFR §416.54
Effective Date	[DATE]
Last Reviewed	[DATE]
Next Review Due	[DATE + 12 months]
Approved By	Governing Body — Date: ____

Purpose

To establish a comprehensive emergency preparedness plan that addresses the facility's response to natural disasters, man-made emergencies, and public health emergencies, as required by CMS Emergency Preparedness Rule.

Policy Statement

[FACILITY NAME] maintains an emergency preparedness plan based on a facility-specific, community-based risk assessment. The plan is reviewed and updated annually, and exercises are conducted at least twice per year.

Required Elements (per CMS §416.54)

1. Risk Assessment & Emergency Planning

- Facility-specific hazard vulnerability analysis (HVA) conducted annually
- Plan addresses: natural hazards (earthquake, tornado, flood, hurricane, winter storm), technological hazards (power failure, water loss, IT failure, HVAC failure), human-caused events (active shooter, bomb threat, workplace violence), public health emergencies (pandemic, bioterrorism)
- Plan includes procedures for safe evacuation and shelter-in-place
- Plan addresses the needs of patients with functional and access needs

2. Communication Plan

- Contact information for: all staff, local emergency management, hospital transfer partners, utility companies, vendors, state/local health departments
- Process for notifying patients' emergency contacts
- Method of communication when primary systems are unavailable (backup phone, satellite phone, radio)
- System for tracking patients during an evacuation

3. Policies & Procedures

- Safe evacuation procedures (horizontal and vertical if applicable)
- Shelter-in-place procedures
- Patient tracking and accountability
- Procedures for informing state/local emergency management about patient needs
- Procedures for clinical and support activities during emergencies (continuity of operations)
- Arrangements for alternative care sites (transfer agreements)

4. Training & Testing

- All staff receive emergency preparedness training at hire and annually
- Exercises conducted twice per year:
 - One full-scale or functional exercise
 - One exercise may be an additional exercise, tabletop, or participation in a community-wide exercise
- If the facility experiences an actual emergency that requires activation of the plan, it counts as one exercise
- After-action reports are completed following each exercise with corrective actions
- Exercises test different scenarios over time (not the same drill repeatedly)

Documentation

- Emergency preparedness plan (complete document)
 - Annual HVA / risk assessment
 - Communication plan with current contact lists
 - Training records (all staff)
 - Exercise documentation (after-action reports with corrective actions)
 - Annual plan review and update documentation
-

Policy 11: Malignant Hyperthermia Response

[FACILITY NAME] — Policy & Procedure

Field	Details
Policy Title	Malignant Hyperthermia Response
Policy Number	EMRG-002
Department	OR / Anesthesia
CMS Reference	42 CFR §416.42(b)
Effective Date	[DATE]
Last Reviewed	[DATE]
Next Review Due	[DATE + 12 months]
Approved By	Governing Body — Date: ____

Purpose

To establish an immediate response protocol for malignant hyperthermia (MH) crisis events in the operating room.

Policy Statement

[FACILITY NAME] maintains an MH-specific response protocol, a dedicated MH cart/kit with Dantrolene, and trains all OR and anesthesia staff at least annually on MH recognition and response.

Procedure

1. Recognition — Early Warning Signs

- Unexplained rise in end-tidal CO₂ (ETCO₂)
- Unexplained tachycardia
- Muscle rigidity (especially masseter muscle rigidity after succinylcholine)
- Rising body temperature (may be a LATE sign)
- Metabolic acidosis, hyperkalemia
- Dark-colored urine (myoglobinuria)

2. Immediate Response — Assign Roles

- **Anesthesia Provider:** Discontinue all triggering agents immediately (volatile agents, succinylcholine); hyperventilate with 100% O₂ at high flows; call for help; begin Dantrolene preparation
- **Circulating Nurse 1:** Bring MH cart to OR; call MHAUS Hotline: **1-800-644-9737**; begin mixing Dantrolene
- **Circulating Nurse 2 / Available Staff:** Obtain ice, cold saline; prepare cooling measures; draw labs (ABG, electrolytes, CK, myoglobin, coagulation panel, urine myoglobin)
- **Surgeon:** Conclude or abort procedure as rapidly as safely possible

3. Dantrolene Administration

- Initial dose: 2.5 mg/kg IV push
- Repeat 1 mg/kg every 5–10 minutes until symptoms resolve (no maximum dose in crisis)

- Mix each 20 mg vial with 60 mL sterile water (or per product formulation — check your supply)
- Continue Dantrolene for at least 24 hours post-crisis (1 mg/kg every 4–6 hours)

4. Cooling Measures

- Cold IV saline (NOT through blood warmer)
- Ice packs to groin, axillae, neck
- Cooling blanket if available
- Stop cooling at 38.5°C (101.3°F) to prevent overcooling

5. Transfer

- Initiate transfer to nearest hospital with ICU capability
- Notify receiving facility of MH crisis
- Send all documentation and lab results with patient

6. MH Cart/Kit Contents & Checks

- Dantrolene: minimum 36 vials (or per MHAUS guidelines for your facility's patient population)
- Sterile water for reconstitution
- Large-bore IV supplies, cold saline
- Cooling supplies
- Lab tubes for required blood draws
- MHAUS Hotline number posted on cart
- Cognitive aid / MH protocol checklist posted on cart
- Cart checked monthly (documented) for expiration dates and completeness

Policy 12: Fire Safety & Response

[FACILITY NAME] — Policy & Procedure

Field	Details
Policy Title	Fire Safety & Response
Policy Number	EMRG-003
Department	Facility-Wide
CMS Reference	42 CFR §416.44 / NFPA 101
Effective Date	[DATE]
Last Reviewed	[DATE]
Next Review Due	[DATE + 12 months]
Approved By	Governing Body — Date: ____

Purpose

To establish fire prevention, detection, and response procedures that protect patients, staff, and visitors.

Policy Statement

All staff are trained in fire prevention and the RACE/PASS response protocol. Fire drills are conducted quarterly with documentation. Fire detection and suppression systems are maintained per NFPA standards.

Procedure

1. Fire Prevention

- No smoking on facility premises
- Electrical equipment is inspected per schedule; no unapproved personal appliances
- Extension cords are not used as permanent wiring
- Storage does not block electrical panels (36-inch clearance), sprinkler heads (18-inch clearance), or egress paths
- Oxygen storage follows NFPA 99 requirements
- Fire-resistant construction materials maintained

2. Fire Response — RACE Protocol

- **R — Rescue:** Remove patients/persons in immediate danger
- **A — Alarm:** Activate the nearest fire alarm pull station; call 911
- **C — Contain:** Close all doors to contain fire and smoke
- **E — Extinguish/Evacuate:** If small and contained, use fire extinguisher (PASS); if large, evacuate

3. Fire Extinguisher Use — PASS

- **P — Pull** the pin
- **A — Aim** at the base of the fire
- **S — Squeeze** the handle
- **S — Sweep** side to side

4. Evacuation

- Primary and secondary evacuation routes are posted in every department
- Patients in OR are stabilized before evacuation if possible; anesthesia provider manages airway
- Wheelchair and stretcher patients are evacuated per trained technique
- Assembly point: **[SPECIFY LOCATION]**
- Staff accountability is verified at assembly point

5. Fire Drills

- Conducted quarterly (minimum) on varying shifts and days
- At least one drill per year during hours of operation with patients present (or simulated)
- Documentation includes: date, time, shift, participants, scenario, response time, findings, corrective actions

6. Fire System Maintenance

- Fire alarm system: inspected and tested annually by qualified vendor
 - Sprinkler system: inspected quarterly (visual) and annually (full inspection)
 - Fire extinguishers: monthly visual inspection, annual professional inspection
 - Emergency lighting: monthly 30-second test, annual 90-minute test
 - Fire doors: inspected annually for proper operation
-

Policy 13: Medical Staff Credentialing & Privileging

[FACILITY NAME] — Policy & Procedure

Field	Details
Policy Title	Medical Staff Credentialing & Privileging
Policy Number	CRED-001
Department	Administration / Medical Staff
CMS Reference	42 CFR §416.45
Effective Date	[DATE]
Last Reviewed	[DATE]
Next Review Due	[DATE + 12 months]
Approved By	Governing Body — Date: ____

Purpose

To establish a standardized process for credentialing and granting clinical privileges to physicians and other practitioners who provide care at **[FACILITY NAME]**.

Policy Statement

All practitioners who perform surgical procedures or administer anesthesia at **[FACILITY NAME]** must be credentialed and granted specific clinical privileges by the Governing Body. Privileges are based on verified education, training, experience, current competency, and ability to perform the requested procedures.

Procedure

1. Application Process

- Applicant completes the facility's credentialing application
- Application includes: personal information, education, training, board certification, licensure, DEA registration, malpractice history, malpractice insurance, hospital privileges, references, and requested privileges

2. Primary Source Verification (Required)

- Medical school graduation (or equivalent)
- Residency/fellowship completion
- Current state medical license (verified with state board)
- Board certification status (verified with certifying board)
- DEA registration (verified with DEA)
- NPDB query (initial and at each reappointment)
- Malpractice insurance (current certificate of insurance)
- Hospital privilege status (verification letter)
- Professional references (minimum of 2 peer references)

3. Privilege Delineation

- Privileges are procedure-specific (not blanket “surgery” privileges)
- Privilege request forms list specific procedures by category
- Criteria for granting privileges are based on training, experience, current competency, and facility capability
- New or expanded privileges require additional documentation of training/proctoring

4. Governing Body Approval

- Completed application with all verifications is presented to the Governing Body (or designated committee)
- Governing Body grants, denies, or tables the application
- Decision is communicated to the applicant in writing
- Initial appointment may include a provisional period with proctoring requirements

5. Reappointment

- Reappointment cycle: every **[2 years — per facility bylaw; CMS minimum is every 2 years for AAAHC]**
- Reappointment considers: current license, insurance, NPDB, peer review data, quality outcomes, compliance with facility policies, volume of cases
- Updated primary source verification at each reappointment

6. Temporary Privileges

- Granted only when there is an important patient care need
- Requires: complete application submitted, verification of current license, current DEA, current malpractice insurance
- Limited to a defined period (typically 120 days maximum)
- Approved by the Medical Director or Governing Body chair

Credential File Checklist (Per Provider)

Document	On File?	Verified?	Expiration
Completed application	☐	N/A	N/A
Current state medical license	☐	☐	
DEA registration	☐	☐	
Board certification verification	☐	☐	
Medical education verification	☐	☐	N/A
Residency/fellowship verification	☐	☐	N/A
Malpractice insurance certificate	☐	☐	
NPDB query (within 2 years)	☐	N/A	
Hospital privilege verification	☐	☐	
Peer references (2 minimum)	☐	N/A	N/A
Privilege delineation (signed)	☐	N/A	N/A
Governing Body approval documentation	☐	N/A	N/A
Malpractice history disclosure	☐	N/A	N/A
Photo ID	☐	N/A	N/A
TB screening (if required by policy)	☐	N/A	

Policy 14: Peer Review & Ongoing Professional Practice Evaluation

[FACILITY NAME] — Policy & Procedure

Field	Details
Policy Title	Peer Review & Ongoing Professional Practice Evaluation (OPPE)
Policy Number	CRED-002
Department	Medical Staff
CMS Reference	42 CFR §416.45
Effective Date	[DATE]
Last Reviewed	[DATE]
Next Review Due	[DATE + 12 months]
Approved By	Governing Body — Date: ____

Purpose

To establish a systematic process for evaluating the professional performance of all credentialed practitioners on an ongoing basis, supporting quality care and informed reappointment decisions.

Policy Statement

[FACILITY NAME] conducts ongoing professional practice evaluation (OPPE) for all practitioners with clinical privileges. Peer review is conducted for all adverse events, complications, and as part of routine quality monitoring.

Procedure

1. Ongoing Professional Practice Evaluation (OPPE)

- Data is collected for each practitioner at minimum every 6 months
- Indicators include (as applicable):
- Surgical site infection rates
- Unplanned transfers to hospital
- Return to OR (unplanned)
- Patient complaints related to the practitioner
- Medical record completion and timeliness
- Compliance with universal protocol
- Compliance with facility policies
- Case volume

2. Focused Professional Practice Evaluation (FPPE)

- Triggered by:
- New privilege requests
- Initial appointment (provisional period)

- Concerns identified through OPPE, incident reports, or complaints
- FPPE includes proctoring, chart review, or direct observation
- Duration and requirements defined at initiation

3. Peer Review Process

- Cases are reviewed by a physician of the same or similar specialty
- Review categories: standard of care met / opportunity for improvement / deviation from standard of care
- All reviews are confidential and protected under state peer review privilege statutes
- Results are communicated to the practitioner
- Trends are reported to QAPI and Governing Body (de-identified aggregate data)

4. Corrective Action

- If peer review identifies practice concerns:
 - a. Educational counseling (documented)
 - b. Focused monitoring with defined criteria
 - c. Mandatory proctoring
 - d. Privilege restriction or suspension
 - e. Privilege revocation (requires Governing Body action and fair hearing process per bylaws)

Policy 15: Quality Assessment & Performance Improvement (QAPI)

[FACILITY NAME] — Policy & Procedure

Field	Details
Policy Title	Quality Assessment & Performance Improvement (QAPI)
Policy Number	QA-001
Department	Facility-Wide
CMS Reference	42 CFR §416.43
Effective Date	[DATE]
Last Reviewed	[DATE]
Next Review Due	[DATE + 12 months]
Approved By	Governing Body — Date: ____

Purpose

To establish a data-driven Quality Assessment and Performance Improvement program that identifies opportunities for improvement and implements changes that enhance patient safety and quality of care.

Policy Statement

[FACILITY NAME] maintains an active QAPI program that is Governing Body-directed, data-driven, and involves all departments and staff. The program measures, assesses, and improves processes related to patient health outcomes and patient safety.

Procedure

1. QAPI Program Structure

- **Governing Body:** Has ultimate responsibility; reviews QAPI reports quarterly; approves QAPI plan annually
- **QAPI Committee:** Meets quarterly (minimum); includes Administrator, Medical Director, nursing leadership, IC Officer, and quality designee
- **QAPI Coordinator:** Day-to-day management of data collection and reporting

2. Required Quality Indicators (Minimum)

- Surgical site infection rates
- Patient falls
- Unplanned hospital transfers
- Unplanned returns to OR
- Medication errors / adverse drug events
- Wrong site/wrong side/wrong patient events
- Patient satisfaction scores
- Compliance with surgical safety checklist (time-out)

3. Performance Improvement Projects

- Minimum of 2 active PI projects at all times
- At least 1 project must be clinical; at least 1 must address patient safety
- Projects use a structured methodology: Plan-Do-Study-Act (PDSA) or equivalent
- Each project has: defined aim statement, baseline data, interventions, re-measurement, and sustainability plan

4. Data Collection & Analysis

- Data is collected monthly for all quality indicators
- Data is analyzed quarterly for trends, benchmarks, and statistical significance
- Data is presented to QAPI committee quarterly and Governing Body quarterly

5. Annual QAPI Program Evaluation

- The effectiveness of the entire QAPI program is evaluated annually
- Evaluation includes: were goals met? What projects were completed? What outcomes improved?
- Next year's QAPI plan is developed based on the evaluation

Documentation

- QAPI program description
 - QAPI committee meeting minutes (quarterly)
 - PI project documentation (each project — aim, data, interventions, results)
 - Governing Body review of QAPI (quarterly minutes)
 - Annual QAPI program evaluation
-

Policy 16: Incident Reporting & Investigation

[FACILITY NAME] — Policy & Procedure

Field	Details
Policy Title	Incident Reporting & Investigation
Policy Number	QA-002
Department	Facility-Wide
CMS Reference	42 CFR §416.43
Effective Date	[DATE]
Last Reviewed	[DATE]
Next Review Due	[DATE + 12 months]
Approved By	Governing Body — Date: ____

Purpose

To establish a standardized process for reporting, investigating, and resolving incidents, adverse events, and near-miss events to improve patient safety and prevent recurrence.

Definitions

- **Incident:** Any event that is not consistent with routine care or operations that did or could result in harm
- **Adverse Event:** An incident that results in patient harm
- **Near-Miss:** An event that could have resulted in harm but was caught before reaching the patient
- **Sentinel Event:** An unexpected occurrence involving death or serious physical/psychological injury

Procedure

1. Reporting

- Any staff member who witnesses or becomes aware of an incident must complete an incident report within 24 hours
- Reports are submitted to the Administrator / Risk Manager
- Reporting is non-punitive — the focus is system improvement, not individual blame
- Near-misses are reported using the same process

2. Immediate Response

- Ensure patient safety and provide necessary treatment
- Notify the attending physician immediately
- Preserve any physical evidence
- Document factual observations in the medical record (NOT the incident report)
- The incident report is NOT part of the medical record

3. Investigation

- All incidents are reviewed by the Administrator within 48 hours

- Serious events or sentinel events trigger a root cause analysis (RCA)
- Investigation includes: timeline of events, contributing factors, interviews, record review, system analysis
- RCA identifies root causes (not just proximate causes) and recommends corrective actions

4. Corrective Actions

- Corrective action plans are developed for all identified root causes
- Actions are specific, measurable, and assigned to a responsible party with a deadline
- Effectiveness of corrective actions is monitored

5. External Reporting

- Sentinel events are reported per state requirements and accreditor requirements
- Device-related adverse events are reported to FDA MedWatch
- Reportable infections are reported to the state health department

6. Trending & QAPI Integration

- All incidents are logged and trended monthly
- Incident data is reviewed by QAPI committee quarterly
- Patterns trigger performance improvement projects

Policy 17: Medical Records Management

[FACILITY NAME] — Policy & Procedure

Field	Details
Policy Title	Medical Records Management
Policy Number	DOC-001
Department	Facility-Wide
CMS Reference	42 CFR §416.47
Effective Date	[DATE]
Last Reviewed	[DATE]
Next Review Due	[DATE + 12 months]
Approved By	Governing Body — Date: ____

Purpose

To ensure that medical records are accurately maintained, readily available, and securely stored in compliance with CMS, HIPAA, and state regulations.

Policy Statement

[FACILITY NAME] maintains a complete, accurate, and timely medical record for each patient. Records are organized, securely stored, and accessible to authorized individuals.

Required Medical Record Contents (per CMS §416.47)

Each patient record must contain:

1. Patient identification information (name, DOB, address, emergency contact)
2. Significant medical history and results of physical examination
3. Pre-operative diagnostic studies and their results
4. Allergies and drug sensitivities
5. Informed consent (signed, per Policy PR-002)
6. Advance directive information (documented)
7. Pre-anesthesia assessment
8. Intra-operative anesthesia record
9. Name of the physician who administered or supervised anesthesia
10. Name of the physician who performed the procedure
11. Operative report (dictated or written immediately post-procedure)
12. Pathology reports (when specimens obtained)
13. Post-anesthesia evaluation (recovery assessment)
14. Discharge notes including condition at discharge, discharge instructions, and follow-up plan
15. Physician orders (signed or authenticated)
16. All medication administration records
17. Nursing assessments and notes

Procedure

1. Record Completion

- Records are initiated at the time of admission
- All entries are dated, timed, and authenticated (signature, initials, or electronic authentication)
- Abbreviations follow the facility's approved abbreviation list; "Do Not Use" abbreviations are prohibited
- Operative report: completed immediately post-procedure; if dictated, a brief operative note is written in the chart until transcription is available
- All records are completed within 30 days of discharge

2. Record Security & Confidentiality

- Medical records are stored in a secure, lockable area or within a HIPAA-compliant electronic system
- Access is limited to authorized personnel
- Release of records follows HIPAA authorization requirements

3. Record Retention

- Records are retained for the period required by state law (minimum): **[INSERT STATE REQUIREMENT — commonly 7-10 years for adults; until age of majority + state requirement for minors]**
- If the facility closes, records are transferred to an appropriate custodian

4. Record Audits

- Quarterly medical record audits are conducted (minimum 10 records per quarter)
 - Audit results are reported to QAPI committee
 - Deficiencies in record completion are addressed through provider-specific feedback
-

Policy 18: Discharge Planning & Follow-Up

[FACILITY NAME] — Policy & Procedure

Field	Details
Policy Title	Discharge Planning & Follow-Up
Policy Number	PC-001
Department	Clinical
CMS Reference	42 CFR §416.52
Effective Date	[DATE]
Last Reviewed	[DATE]
Next Review Due	[DATE + 12 months]
Approved By	Governing Body — Date: ____

Purpose

To ensure patients are safely discharged from [FACILITY NAME] with clear instructions and appropriate follow-up arrangements.

Policy Statement

No patient is discharged without meeting established discharge criteria, receiving a physician's discharge order, and receiving written discharge instructions. A responsible adult must be available to accompany the patient who has received sedation or anesthesia.

Discharge Criteria

Patients must meet the following criteria before discharge (based on modified Aldrete scoring or facility-equivalent):

Criterion	Requirement
Level of consciousness	Alert and oriented (or returned to pre-procedure baseline)
Vital signs	Stable for minimum 30 minutes; within acceptable parameters
SpO2	≥ 92% on room air (or at pre-procedure baseline)
Pain	Controlled to an acceptable level (patient-reported)
Nausea/Vomiting	Controlled or absent
Surgical site	No active bleeding; dressing dry and intact
Ambulation	Able to ambulate with minimal assistance (or at baseline)
Voiding	Per facility/physician criteria based on procedure type
Responsible adult	Present and identified

Procedure

1. Discharge Assessment

- Registered nurse assesses patient against discharge criteria
- Physician authorizes discharge (written or verbal order)
- Any deviation from standard discharge criteria is documented with physician justification

2. Discharge Instructions

Written discharge instructions are provided and reviewed with the patient AND responsible adult, including:

- Procedure performed and name of physician
- Post-operative care instructions specific to the procedure
- Activity restrictions and duration
- Diet instructions
- Medication instructions (new prescriptions, changes to home medications)
- Pain management plan
- Signs and symptoms requiring immediate medical attention
- When and how to contact the surgeon
- Follow-up appointment information
- Emergency contact numbers (facility, surgeon, 911)

3. Documentation

- Patient and responsible adult sign acknowledgment of receiving discharge instructions
- Copy of instructions given to patient; copy filed in medical record
- Discharge time, condition at discharge, mode of transport, and responsible adult documented

4. Post-Discharge Follow-Up

- **[FACILITY NAME]** conducts a post-discharge follow-up contact within 24–72 hours
 - Follow-up assesses: pain level, signs of infection or complications, adherence to instructions, patient satisfaction
 - If the patient reports a concern, the surgeon is notified and appropriate action is taken
 - Follow-up call is documented in the patient's medical record
-

Policy Review Schedule

Policy #	Policy Title	Review Frequency	Last Review	Next Review	Reviewer
IC-001	Infection Control Program	Annual	[DATE]		IC Officer
IC-002	Hand Hygiene	Annual	[DATE]		IC Officer
IC-003	Sterilization & HLD	Annual	[DATE]		IC Officer / SPD Lead
PHARM-001	Medication Management	Annual	[DATE]		Clinical Lead
PHARM-002	Controlled Substance Management	Annual	[DATE]		Clinical Lead / Admin
PR-001	Patient Rights	Annual	[DATE]		Administrator
PR-002	Informed Consent	Annual	[DATE]		Clinical Lead
PR-003	Advance Directives	Annual	[DATE]		Clinical Lead
PR-004	Grievance Resolution	Annual	[DATE]		Administrator
EMRG-001	Emergency Preparedness	Annual	[DATE]		Administrator
EMRG-002	Malignant Hyperthermia	Annual	[DATE]		Anesthesia Lead
EMRG-003	Fire Safety	Annual	[DATE]		Safety Officer
CRED-001	Credentialing & Privileging	Annual	[DATE]		Administrator
CRED-002	Peer Review / OPPE	Annual	[DATE]		Medical Director
QA-001	QAPI	Annual	[DATE]		Quality Lead

Policy #	Policy Title	Review Frequency	Last Review	Next Review	Reviewer
QA-002	Incident Reporting	Annual	[DATE]		Administrator
DOC-001	Medical Records	Annual	[DATE]		Compliance Lead
PC-001	Discharge Planning	Annual	[DATE]		Clinical Lead

Document Version Control

Version	Date	Author	Changes
1.0	Initial Release	ASC Compliance Team	Original 18-policy template library

This document is part of the ASC Compliance Manual — Tier 2: Complete Certification System (\$197). Includes all Tier 1 content plus Staff Training Checklist, Policy Template Library, and Quarterly Update Guide.